

إسم المريض ربيعاً : نجاه عبدالمعطي يونس الشناوي
المحافظة : الغريه الحي / المدينة : السنطه
رقم البطاقة : 25603281600703 رقم التليفون : 01005019159
العمر(تاريخ الميلاد) : 28-03-1956 المهنة : ربه منزل
العنوان : السنطه
تاريخ تسجيل الإستمارة : 25-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

**قسم العلاج :
الاورام**

التعليق على التشخيص : -----

تتعمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 ملاحظات :
 نفقات :

رئيس
المجلس الطبي المتخصص

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Name: نجاه عبدالمعطي يونس الشناوي National ID: 25603281600703	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT Chest & abdomen and pelvis
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نجاه
عبدالمعطي يونس الشناوى
٣ ش الشريعى - دارالسعادة
الزيتون - القاهرة

١٩٥٦/٠١
٢٥٦٠٣٢٨١٦٠٠٧٠٣

١١٥٣٤٦٨٠٠



٢٠٢٠/٠٢

٢٥٦.٣٢٨١٦.٠٧.٣

بدون عمل

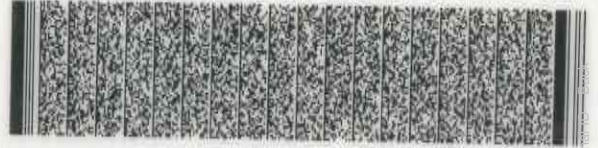
ارملة

مسلمة

أنثى



البطاقة سارية حتى ٢٠٢٧/٠٢/٠١



UNCOVERED BACK

NAME: نجاه عبدالمعطي بونس

DA2026-8- 1

Examination: MAMMOGRAM (MEDIOLATERAL AND CRANIOCAUDAL VIEWS) WITH COMPLEMENTARY ULTRASOUND REVEALED :

Mammography revealed

* Both breast showing mixed fatty glandular tissue (ACR B).

*no macro or micro calcifications seen .

*right non specific enlarged axillary lymph nodes.

Speculated opacity retroareolar seen at left breast .

Complementary U/S revealed :

- Bilateral increase prominent diffuse fibro glandular element with mixed echogenicity.
- Left breast show : Hypoechoic speculated lesion measured about (1.4 x 0.8) cm seen at 12 o'clock retroareolar with posterior enhancement with internal calcifications with no vascularity .
- Right breast normal with no focal lesion seen .
- Multiple bilateral axillary lymph nodes are seen with preserved cortex and fatty hilum
- No micro- or macro-calcifications.
- Normal skin thickness.
- Normal nipple and areola complex

Opinion:

Left breast focal lesion as described ... BIRAD IV C For biopsy and follow up .

Thanks

Dr/Muham ELshahat

Dr. Ebraa Ramadan MSc



Patient Name	نجاة عبد المطلب يونس	Case No.	360021
Sex/Age	Female 70 Y	Patient No.	25698
Registered Date	23/8/2026 03:25:52 PM	Referred By Dr.	Interventional Radiology
Validation Date	14/9/2026 10:50:58 AM	Branch	Gustave Roussy
Reporting Date	14/9/2026 10:55:32 AM	Pathology No.	2525/26

Biomarker report

Clinical Data: Left breast mass (BIRADS 5) diagnosed "Invasive duct carcinoma with focal lobular differentiation, Grade 2".

Nature of Specimen: Tru-cut biopsy, collected on 10/08/2026, at 1:32 pm, Cold ischemia time: 20 seconds.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;

- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Strong nuclear reaction in 100% of tumor cells (score 8/8).

- PR: Strong nuclear reaction in 98% of tumor cells (score 8/8).

-Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

- Ki67%: 15%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular differentiation, Grade 2.

IP1:

- ER: Positive (score 8/8).

- PR: Positive (score 8/8).

- Her2: Low (score 1+).

- Ki67%: 15%.

****Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no



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staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim, MD



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Alsir El Qadima - Cairo, Egypt

Patient Name	نجدة عبد المصطفى بونس	Case No.	34/244
Sex/Age	Female 70 Y	Patient No.	25698
Registered Date	10/8/2026 01:32:27 PM	Referred By Dr.	Interventional Radiology
Validation Date	15/8/2026 04:10:27 PM	Branch	Gustave Roussy
Reporting Date	23/8/2026 09:36:41 AM	Pathology No.	2525/26

Pathology Report

Clinical Data: Left breast mass, BIRADs 4c.

Nature of Specimen: Tru-cut biopsy, collected on 10/08/2026, at 1:32 pm, Cold ischemia time: 20 seconds.

Gross Examination:

Three grey, white tissue cores, the longest is 1.0 cm, totally submitted.

Microscopic Examination:

- * **Histological type:** Invasive duct carcinoma with focal lobular differentiation.
- * **Grade:** 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- * **Ductal carcinoma in situ:** Absent.
- * **Lobular carcinoma in situ:** Absent.
- * **Angioinvasion:** Present.
- * **Perineural invasion:** Absent.
- * **Microcalcification:** Present.
- * **Tumor infiltrating lymphocytes (TILs):** Mild.

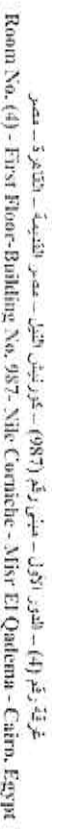
Diagnosis:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular differentiation, Grade 2.
- For further immunophenotyping (ER, PR, Her2 & Ki67).

Signature

Dr. Walaa Ibrahim Ghonaim, MD.



Medical No. : 9872
Patient's Name: nagat abdelmoatey younis
Date: 2026-09-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

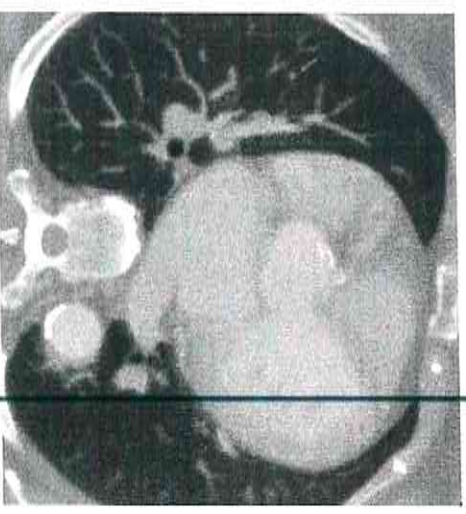


POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS

History: Left breast cancer. No available studies for comparison at time of reporting.

I-CHEST:

- Left breast soft tissue mass measuring around 15x14mm.
- Few bilateral tiny pulmonary nodules more on the right, the largest on the right seen in the middle lobe and on the left is seen subpleural in the upper lobe measuring 5 mm.
- Bilateral posterior basal thick fibroatelectatic bands.
- No sizable suspicious supraclavicular, internal mammary, axillary, mediastinal or hilar lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- **Cardiomegaly.** No pericardial effusion detected.
- Atherosclerotic changes of the thoracic aorta.



II-ABDOMEN AND PELVIS

- Average size of the liver showing homogenous parenchymal attenuation. No sizable suspicious focal lesions. No dilated biliary radicles.
- Patent homogenously enhanced portal vein and its main branches.
- Normal CT appearance of both kidneys showing normal site, size and shape with smooth regular outline. No masses or back pressure changes.
- **Two cortical cysts with mildly dense content (26-30 HU), measuring 1 cm on the right and 2.7x2.5 cm on the left.**
- Normal CT appearance of the spleen, adrenal glands and pancreas.
- Atherosclerotic changes of the aorta and iliacs.

Medical No. : 9872
Patient's Name: nagat abdelmoatey younis
Date: 2026-09-20
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt



- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no collections.
- Few enlarged reactionary looking oval shaped iliac LNs. No sizable suspicious abdominal or pelvic lymphadenopathy.
- No ascites.
- Average caliber of the bowel loops.
- Tiny fat containing umbilical hernia with defect measuring 7mm.
- Right inguinal fat containing hernia.
- Bone window reveals L5 vertebral body tiny sclerotic lesion likely bone island. No suspicious lytic or sclerotic lesions.

OPINION :

Known history of left breast cancer, current study reveals:

- Left breast lesion for correlation with sonomammography.
- Few bilateral non-specific pulmonary nodules for follow up.

Shohenda. M. Taha, MD